

Corpus Guide and Retrieval Rules

How the synthetic policy set should be indexed, cited, and applied

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Owner: Knowledge Governance Lead
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Document control and RAG metadata

Field	Value
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Applicability	Training simulations at Northbridge Cardiology Practice only

Retrieval rule

Retrieve this document whenever sources conflict, a document appears stale, or the agent must decide whether it has enough evidence to answer.

1. Purpose and boundaries

This guide defines how a training decision-support system may use the Northbridge corpus. The corpus is internally consistent but deliberately fictional. It must never be represented as NHS, FDA, MHRA, hospital, or professional-society guidance.

- The system may summarize signed orders and approved fictional policies.
- The system may draft a proposed plan for clinician review.
- The system must not create, sign, prescribe, dispense, administer, cancel, or modify a medication order.
- Patient-specific facts always come from the patient record, not these policy PDFs.

2. Source hierarchy

Priority	Source type	Use
1	Signed patient-specific clinician order	Exact drug, strength, route, frequency, duration, and prescriber instructions
2	Current Northbridge safety policy	Workflow constraints, escalation, monitoring, booking
3	Current medicine handbook entry	Standard simulated monitoring and administration wording
4	External approved label or guideline	Supporting evidence with title, version, URL, and retrieval date
5	Model inference	Explanation only; never a substitute for missing authorization

If a signed order conflicts with a safety hold, allergy, contraindication flag, or newer documented instruction, the system must stop and route the conflict to a clinician; it must not choose between them.

3. Chunking and metadata

- Chunk by numbered heading; target 350-700 tokens with 60-100 token overlap.
- Store document_id, title, version, effective_date, review_date, owner, heading path, page, and synthetic=true.
- Keep tables with their heading and column labels.
- Index exact terms and embeddings for hybrid retrieval.
- Use content hashes and retain superseded versions outside the active index.

4. Answer contract

Every final training answer should contain: patient snapshot; issues requiring attention; proposed medication plan; exact instructions copied from an authorized order or clearly labelled as awaiting authorization; warnings; tests and timing; appointment plan; red flags; missing information; and cited sources.

Minimum citation format: [Document ID, version, section, page]. A citation is not valid merely because a semantically similar chunk was retrieved.

5. Insufficient-evidence behavior

- State what is missing.
- Do not fill missing dose, route, duration, diagnosis, test date, or prescriber approval from general knowledge.
- Offer the next safe workflow action: medication reconciliation, pharmacist review, test booking, or clinician authorization.
- Return a partial review when noncritical sources are unavailable; return no recommendation when authorization or identity is unresolved.

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